

Announcement

Kindly note: We're currently experiencing a high volume of inquiries and calls, potentially leading to delays in our initial response.



Dermatology 23S Module 2

[Dashboard](#) / [My courses](#) / [Dermatology 23S Module 2](#) / [Week 2 - Benign skin lesions](#) / [Go to Week 2 Academic Forum](#)
/ [Scenario 3](#)

 Search forums

Go to Week 2 Academic Forum

Scenario 3

Settings ▾

[◀ Day 2-3 Scenario 2](#) Display replies in nested form**Scenario 3**

by [Derek Kavanja \(Tutor\)](#) - Tuesday, 14 November 2023, 2:22 PM

Hi Group,

Here is our last scenario this week (there shall be some follow on questions).

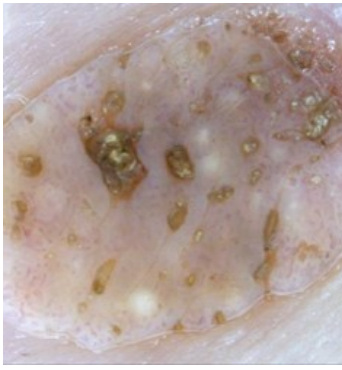
A 55-year-old man attends clinic with a pigmented lesion in the mid back region. His wife noticed this and was concerned about it. She feels that there has been a change in the size and colour of this lesion over the last 6 months. The patient reports the occasional sensation of itching. He is a keen sailor and spends a lot of time outdoors. There is no other history of significance.

This is the appearance of the lesion:



Please answer all the questions below:

1. What bedside test would be useful in confirming the diagnosis?
2. This is the lesion under magnification:



What is your diagnosis?

3. What is the most appropriate management strategy for this patient?

131 words

Reply



Re: Scenario 3

by [Zarmeena Khan](#) - Tuesday, 14 November 2023, 7:03 PM

Thank you for the clinical scenario,

As for questions 1 and 2,

The lesion in question looks similar to seborrheic keratosis.

It's a common non-cancerous skin growth that appears as a wart-like or stuck-on lesions, these can be varied in colour from yellow to grey to brown and black.

These can have a smooth /waxy or warty surface.

(Dermnetnz.org, 2022)

The diagnosis of seborrheic keratosis is typically based on clinical evaluation. But in my opinion, dermatoscopy would be a good non-invasive diagnostic technique that allows a closer examination of the skin, and can be used to further assess the characteristics of seborrheic keratoses. It may reveal features like the "church spire" pattern, comedo-like openings, and milia-like cysts. Blood vessels can be prominent, which are mostly hairpin-shaped capillaries surrounded by a halo within a lobule.

3. Treatment options:

It is a benign condition and treatment is usually not necessary unless there are cosmetic or symptomatic concerns. If treatment is desired, several options are available. Common management strategies for seborrheic keratosis include:

1. Observation:

- In many cases, especially if the seborrheic keratosis is asymptomatic and does not cause cosmetic concerns, the best approach may be to leave it untreated and monitor it over time.

2. Cryotherapy:

- Cryotherapy involves freezing the seborrheic keratosis with liquid nitrogen. This method is commonly used for smaller lesions and may cause the growth to fall off over time.

3. Curettage:

- Curettage involves scraping off the seborrheic keratosis using a spoon-shaped instrument called a curette. This method is often combined with electrosurgery to cauterize the base of the lesion.

4. Laser Therapy:

- Laser therapy can be used to remove seborrheic keratoses. Different types of lasers, such as CO2 or erbium: YAG lasers, may be employed for this purpose.

5. Topical Treatments:

- Some topical treatments, such as certain acids or creams containing glycolic acid, may be used to gradually reduce the thickness of the lesion.

6. Shave Excision:

- Shave excision involves shaving off the raised part of the seborrheic keratosis using a scalpel.

For this patient in particular, if he is having issues with itching, cosmetic reasons or the lesion getting caught in clothes, I would recommend removal by shave excision/curettage or cryo therapy.

-Questions for the group, are there any complications of Seborrheic keratosis that we as clinicians should be aware of?

Reference:

1.Gupta, A. K., & Paquet, M. (2007). *Network meta-analysis of the outcome "complete clearance" in topical treatment of actinic keratosis*. Journal of the American Academy of Dermatology, 56(2), 274-281. <https://doi.org/10.1016/j.jaad.2006.09.028>

2.Dermnetnz.org. (2022). *Dermoscopy. Seborrhoeic keratoses / DermNet*. [online] Available at: <https://dermnetnz.org/cme/dermoscopy-course/dermoscopy-of-seborrhoeic-keratosis> [Accessed 14 Nov. 2023].

3.Moncrieff, M. (2014). Management of seborrhoeic keratosis by shave excision in the outpatient setting. International Journal of Dermatology, 53(12), 1452-1453.

4.Lebwohl, M. G. (2006). Medical Pearl: The use of glycolic acid as a peeling agent. Journal of the American Academy of Dermatology, 54(2), 348-349. <https://doi.org/10.1016/j.jaad.2005.08.060>

480 words

Reply



Re: Scenario 3

by [Adam Tollitt](#) - Tuesday, 14 November 2023, 7:56 PM

Hi Zarmeena,

In response to your question regarding complications of Seborrhoeic Keratoses (SKs), the most frequent complication of SKs that I encounter in my primary care practice is associated bleeding and/or pain/discomfort related to localised trauma, e.g. rubbing on clothing.

Sometimes an irritated SK can lead to a localised eczematous dermatitis around the SK lesion (Oakley, 2016).

Another important complication to be aware of with SKs is that sometimes a skin cancer may arise within a SK or coalesce with it, making identification of the secondary malignant lesion challenging. This is important to be aware of/counsel patients about and if there is any concern about rapidly changing lesion appearance or other new features concerning for malignancy then then SK should be biopsied or undergo complete diagnostic excision (Oakley, 2016).

A rarer potential complication of SKs that I read about is the sign of Leser-Trelat (Buckley and Pasquali, 2021). This is a paraneoplastic syndrome that represents a sudden eruption of new SKs that is associated with an underlying internal malignancy, most commonly adenocarcinoma of the breast, stomach, ovaries or uterus.



Figure 1 – The sign of Leser-Trelat in a man who presented with >500 new seborrheic keratoses (all within 6 months) without any other associated symptoms. Subsequent investigation revealed a moderately differentiated colorectal adenocarcinoma of the sigmoid colon (Daviti and Lallas, 2021).

References

Buckley, D. and Pasquali, P. (2021). *Textbook of Primary Care Dermatology*. Cham, Switzerland Springer.

Daviti, M. and Lallas, A. (2021). Leser-Trélat sign. *British Medical Journal (BMJ)*, p.n1966.
doi:<https://doi.org/10.1136/bmj.n1966>.

Oakley, A. (2016). Seborrheic keratoses (brown warts, basal cell papillomas, seborrheic keratosis). [online] dermnetnz.org. Available at: <https://dermnetnz.org/topics/seborrheic-keratosis>. (accessed 14/11/2023).

268 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:18 PM

Thank you Adam, the sign of Leser-Trelat is closely associated with adenocarcinoma of the stomach and colon.

17 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:01 PM

Thank you Zarmeena, very good post and correct diagnosis.

9 words

Reply



Re: Scenario 3

by [Oyeombiliyetu Jason](#) - Sunday, 19 November 2023, 2:46 PM

Complications of Seborrheic Keratosis:

1. Potential for overlooking premalignant tumors due to:
 - Misdiagnosis, distinguishing between skin cancer and seborrheic keratosis.
 - Presence of skin cancer lesions within seborrheic keratosis lesions.
2. Eruptive seborrheic keratosis associated with:
 - Internal malignancy.
 - Adverse reactions to medication.
3. Site and size-dependent discomfort and irritation caused by the lesions.
4. Psychological impact of seborrheic keratosis lesions.
5. Superimposed infections

References

1. Dr A. Oakley (2016). 'Seborrheic keratosis'. Dermnet. Available at: <https://dermnetnz.org/topics/seborrheic-keratosis> accessed: 18 November 2023.
2. M. Greco et al.(2023). 'Seborrheic keratosis' National Library of Medicine. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK545285/> accessed: 19 November 2023.

99 words

Reply



Re: Scenario 3

by [Sadaf Basharat](#) - Sunday, 19 November 2023, 6:04 PM

Hi Zarmeena

Yes there are multiple complications of seborrheic keratosis, one should be aware of. These include.

1. May mask the presence of underlying malignancies.
2. Discomfort and irritation
3. Psychological effects
4. Super imposed infections.

References:

1. Dr A. Oakley (2016). 'Seborrheic keratosis'. Dermnet. Available at: <https://dermnetnz.org/topics/seborrheic-keratosis> accessed: 18 November 2023.
2. Greco, M.J. and Bhutta, B.S., 2019. Seborrheic keratosis.

57 words

Reply

**Re: Scenario 3**by [Felica Mwinji Namukonde](#) - Wednesday, 15 November 2023, 5:20 PM

The lesion looks like Seborrheic Keratosis

mostly diagnosed based on clinical features it usually presents as a well-demarcated oval lesion with a dull verrucous surface and a stuck on warty well circumscribed appearance mostly appear on the trunk, upper extremities and face. Seborrheic keratosis varies in color, black, dark brown and brown, and sometimes maybe multiple.

Close inspection with hand lens shows presence of dark keratin plugs or horn cysts.

Dermascope shows several milium cysts, fissures and comedo-like openings as well as ridges forming a cerebriform.

Biospy is done if unsure of the diagnosis or suspect malignancy.

on histology seborrheic keratosis shows well-demarcated proliferation of keratinocytes that have character of small keratin-filled cysts.

Management

usually treatment is not required but can be done if they are cosmetic concerns, the following can be done;

Cryotherapy; if lesions are thin or flat (only do cryotherapy if diagnosis is confirmed)

Curettage; after procedure submit specimen for pathology

Electrodesiccation which may be followed by curettage, different types of laser can also be used.

However choice of treatment is based on lesion size, thickness, patient skin type and clinical experience

Group question what really confirms the diagnosis of seborrheic keratosis?

References;

Goldstein A O (2023) *Uptodate* Available at: https://www.uptodate.com/contents/overview-of-benign-lesions-of-the-skin?search=chondrodermatitis%20nodularis%20helicis&source=search_result&selectedTitle=1~2&usage_type=def Accessed (15/11/2023)

205 words

Reply

**Re: Scenario 3**by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:02 PM

Thank you Felica, good post and correct diagnosis

8 words

Reply

**Re: Scenario 3**by [Felica Mwinji Namukonde](#) - Sunday, 19 November 2023, 7:12 PM

Common symptoms and their severity as reported by people .

Symptoms and severity graph

- [Pain](#)

(6)

- [Anxious mood](#)

(6)

- [Fatigue](#)

(6)

- [Depressed mood](#)

(6)

- [Stress](#)

(4)

Severe

Moderate

Mild

None

[https://www.bing.com/search?](https://www.bing.com/search?q=complications+of+seborrheic+keratosis&q=NWU&pg=complications+of+seborrheic+keratosis&sc=9-38&cvid=A4CA7206A70F41BC9AFF337A784C4A12&FORM=QBLH&sp=1&ghc=1&lq=0)

[q=complications+of+seborrheic+keratosis&q=NWU&pg=complications+of+seborrheic+keratosis&sc=9-38&cvid=A4CA7206A70F41BC9AFF337A784C4A12&FORM=QBLH&sp=1&ghc=1&lq=0](https://www.bing.com/search?q=complications+of+seborrheic+keratosis&q=NWU&pg=complications+of+seborrheic+keratosis&sc=9-38&cvid=A4CA7206A70F41BC9AFF337A784C4A12&FORM=QBLH&sp=1&ghc=1&lq=0)

31 words

Reply

**Re: Scenario 3**by [Arwaf Benrawwaf](#) - Monday, 11 December 2023, 5:00 PM

Hello Felicia, thank you for your post.

Regarding your question, the diagnosis of Seborrheic keratosis is usually based on clinical examination and history of the lesion. However, in some cases, a biopsy may be necessary to confirm the diagnosis. According to a study published in the Journal of Cutaneous Pathology, a biopsy can help differentiate Seborrheic keratosis from other skin lesions such as melanoma or basal cell carcinoma. The study suggests that histopathological examination of the lesion is the gold standard for confirming the diagnosis of Seborrheic keratosis.

Reference:

-Weedon's Skin Pathology - 4th Edition. [online] Available at: <https://shop.elsevier.com/books/weedons-skin-pathology/patterson/978-0-7020-5183-8> [Accessed 11 Dec. 2023].

104 words

Reply

**Re: Scenario 3**by [Ayten Sadek](#) - Wednesday, 15 November 2023, 11:23 PM

The most likely diagnosis is seborrhoeic keratosis

The history given (the age and sun exposure) and the characteristic appearance of the lesion as it appear as raised, waxy, or stuck-on growths on the skin. They vary in color, often presenting as tan, brown, or black, and they can range in size

The diagnosis of seborrhoeic keratosis is primarily made through clinical examination due to the characteristic

clinical appearance of "stuck on growths on the skin, with a wart like surface, often hyperpigmented lesions located most commonly on the areas of the body exposed to sun as face, chest, shoulders and back. Close inspection with a hand lens often will demonstrate the presence of "horn cyst" appearance, which consists of a warty surface with multiple keratin-filled cysts and papillomatous projections. Examination with a dermatoscope shows multiple milia like cysts, comedo-like openings, clods, fissures and ridges forming a cerebriform pattern

Biopsy may be necessary if the diagnosis is uncertain or if there is concern for malignancy. So may undergo partial shave or punch biopsy or diagnostic excision.

Histologically, seborrheic keratosis typically shows a well-demarcated proliferation of keratinocytes(hyperkeratosis), papillomatosis, acanthosis with characteristic horn cysts filled with keratin.

The management strategy for seborrheic keratosis involves observation in most cases, especially if the lesions are asymptomatic and not causing cosmetic concerns. However, if there are symptoms or the lesions cause irritation or cosmetic issues, There are various treatment options for their removal such as:

Cryotherapy: This involves freezing of the lesion using liquid nitrogen. It is effective for small to medium sized lesion.

Curettage: where the lesion is scrapped off. This method is effective for raised or thicker lesions

laser therapy: It targets and destroys the cells. Effective for various sizes

Shave biopsy as well can be considered

In this patient I would consider cryotherapy or curettage

Reference:

Oakley, A. (2016).seborrheic keratosis | DermNet NZ. [online] Available at: <https://dermnetnz.org/topics/seborrheic-keratosis> [Accessed 15/11/2023]

316 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:02 PM

Thank you Ayten, good post.

5 words

Reply



Re: Scenario 3

by [Pooran Outar](#) - Saturday, 18 November 2023, 1:53 AM

Hi Sadek,

I agree with your responses to the case scenario, drawing from the article by Okley (2016). To enhance the understanding of treatment options for SK, I would like to expand on the considerations for each treatment options you mentioned. It is important to consider several factors, ensuring a personalized approach, taking into account the size, thickness, and skin type of the lesions, along with clinical suspicion and the patient's personal and family history of skin cancers. The physician's experience also plays a role in decision-making. One common treatment is cryotherapy, utilizing liquid nitrogen or CO2 to freeze and thaw cells, causing cell death. However, there are no strict guidelines for freezing/thawing times. While cryotherapy requires low post-procedure care, it may lead to side effects such as erythema, pain, and bulla formation. Shave excisions, involving the removal of a thin tissue slice containing the lesion, require local anesthesia, and specimens are sent to the lab for pathology. Electrodessication with curettage (C&D) efficiently removes epidermal tissue with a low complication rate, utilizing local anesthesia. Topical agents like gels and creams (e.g., tazarotene, imiquimod, alpha-hydroxy acids, urea, vitamin D analogs, topical H2O2 solution) show promising results, though further research is needed. Laser therapy, using ablative and non-ablative lasers, is another option, but its efficacy requires further study (Greco, Mahabadi and Gossman, 2020); (Oakley, 2016).

Reference:

Oakley, A. (2016). Seborrheic keratoses (brown warts, basal cell papillomas, seborrheic keratosis). [online] dermnetnz.org. Available at: <https://dermnetnz.org/topics/seborrheic-keratosis>.

Greco, M.J., Mahabadi, N. and Gossman, W. (2020). Seborrheic Keratosis. [online] PubMed. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK545285/>.

256 words

Reply

**Re: Scenario 3**by [Ayten Sadek](#) - Sunday, 19 November 2023, 8:45 PM

Thank you Pooran for your reply I think one important aspect of the management is considering treatments side effects and discussing them with the patient given that the treatment should be suited according to the patient's condition and preferences. I have included some of the side effects below In cryotherapy thicker lesions may require multiple cycles to effectively treat the lesions. Additionally, this treatment method does not permit any histologic confirmation of the lesion and should only be for low-risk with low clinical suspicion for malignancy. Side effects may include blistering, swelling and potential pigment changes. The curettage method has low rate of complications but may include temporary pain, scarring and leading to either hyperpigmentation (darker spots) or hypopigmentation (lighter spots). Laser therapy might not completely eliminate the lesion leading to the possibility of its return. Topical treatments can cause skin irritation, redness, erythema or itching at the application site.

Reference:

Bhutta.B.S, Greco.M.J (2023) Seborrheic keratosis | ncbi.nlm.nih.gov. [online] Available at: <https://www.ncbi.nlm.nih.gov/books/NBK545285/>

156 words

Reply

**Re: Scenario 3**by [Rahima Abdulaziz Ahmed](#) - Thursday, 16 November 2023, 1:44 PM

I think the bedside test for seborrheic keratosis involves observing the growths characteristics. Seborrheic keratosis often is a well demarcated light brown flat macule. As they initially grow, they develop a velvety to finely verrucous surface followed by an uneven warty surface with multiple plugged follicles and a dull lackluster appearance. (Balin 2020)

Are typically slow growing epidermal skin tumors (benign) and can increase in thickness over time and they rarely resolve spontaneously. An eruption of seborrheic keratosis may develop after an inflammatory dermatoses (eg eczema , severe sunburn), however there should be a concern if there's a sudden growth /emergence of multiple seborrheic keratoses (as discussed by Adam earlier)

Appear to be more frequent in populations with lighter skin tones (like our patient above) and also in patients > 50yrs of age and become more frequent as one ages. These tumors can generally occur anywhere on the body except for the palms , soles and mucous membranes. Depending on the lesion location , there can be chronic inflammation, which leads to pain , pruritus , erythema and bleeding around the site.

(Bhutta and Greco 2023)

Management of this patient;

Reassurance of the patient/couple , that the lesion is common in people with his skin tone and age ,and the changes the wife noticed are also normal for this particular lesions.

Encourage patient to use a sunscreen and protective clothing's while outdoors .

unless patient is concerned with the cosmetic appearance of the lesion ,I would advise him we observe the lesion for now, unless if there are any secondary /malignant changes (which i will educate patient about), to come back immediately for further investigations.

If concerned about cosmetic appearance , would advice patient we do co2 ablative laser.

Question for the group: What changes should the patient look out for/ malignant changes on the lesion for him to come back for review?

What are the subtypes of seborrheic keratoses and how can one differentiate them?

References:

Greco.J and Bhutta.S (2023) *Seborrheic Keratosis*, StatPearls [online] Available at <https://www.ncbi.nlm.nih.gov/books/NBK545285/> (Accessed on 16/11/23)

Balin. K (2020) *Seborrheic Keratosis* , Medscape (online) Available at <https://emedicine.medscape.com/article/1059477-overview> (Accessed on 16/11/23)

355 words

**Re: Scenario 3**

by [Xiao Liey Ding](#) - Thursday, 16 November 2023, 3:07 PM

Hi Rahima,

Agree with you that the management for the patient is reassurance and avoid further sun exposure if there is no cosmetic concern and continue to monitor the lesion.

As for your questions,

Question no.1: What changes should the patient look out for/ malignant changes on the lesion for him to come back for review?

1. Rapid Growth: If a lesion suddenly grows rapidly, it could be a cause for concern.
2. Changes in Color: An increase in pigmentation, development of irregular colors, or changes in the existing color could be a sign of concern.
3. Bleeding or Itching: If a seborrheic keratosis becomes symptomatic, especially if it starts bleeding, itching, or causing discomfort, it should be evaluated.
4. Irregular Borders: While seborrheic keratoses usually have well-defined borders, changes in the borders or the development of irregularities could be a sign of concern.
5. Ulceration or Crusting: Any signs of ulceration or crusting on the surface of the lesion may indicate a problem.

Question no. 2 :What are the subtypes of seborrheic keratoses and how can one differentiate them?

Seborrheic keratosis exhibits diverse subtypes such as acanthotic, hyperkeratotic, clonal, adenoid, irritated, and melanoacanthoma. These lesions can be intricate, often featuring elements from multiple subtypes in their pathology as mentioned by Greco, M.J, and Bhutta,B.S (2023).

Dermoscopic and pathological features of the subtypes were described by Simionescu, O. (2012). Specific elementary dermoscopic lesions were consistently observed, while others were consistently absent in certain forms.

In instances of the acanthotic form, 100% of cases exhibited the presence of more than one colour, yet a network pattern and irregular, polymorphic pattern were absent in all cases. Crypts and milia cysts were present in over 90% of acanthotic cases.

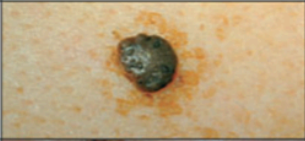
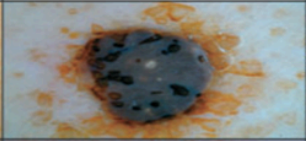
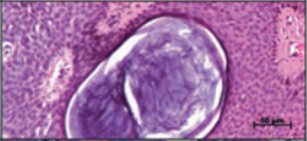
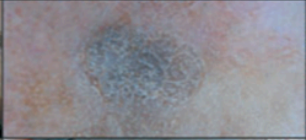
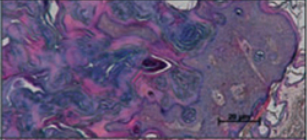
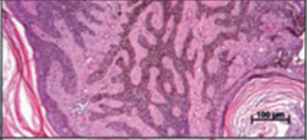
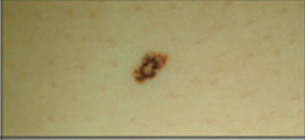
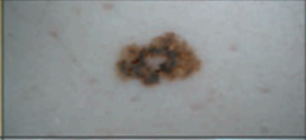
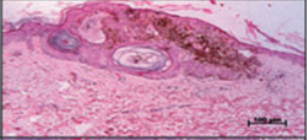
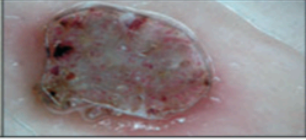
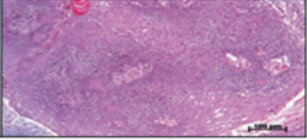
In the hyperkeratotic form, 100% of cases displayed the presence of more than one colour, while a network-like pattern, fat fingers, blue-white pigmentation, and irregular, polymorphic pattern were absent in all cases.

Regarding the reticular form, a network-like pattern, crypts, milia cysts, more than one colour, fissures, and white artifacts were consistently present in 100% of cases, while fingerprinting structures, a network pattern, pinpoint vessels, hairpin vessels, fat fingers, sharp demarcation, and irregular vessels were consistently absent in all cases.

Only two cases of irritated seborrheic keratosis were analyzed in study by Simionescu, O. (2012), and both exhibited an identical dermoscopic picture, including milia cysts, pinpoint vessels, hairpin vessels, more than one colour, white artifacts, and irregular vessels.

In clonal SK, dermoscopy displays variously sized, blue-gray globular structures that cluster to form short lines or are irregularly distributed within the lesion (Bouhamed, M. *et al*, 2019). Additionally, it may manifest other characteristics indicative of seborrheic keratosis (SK), such as well-defined borders, milia-

like cysts, comedo-like openings, and the jelly sign.
Histologically, clonal SK is identified by the presence of well-defined nests of basaloid or pale cells within the epithelium, corresponding to the globular structures observed during dermoscopic examination.

Subtype of seborrheic keratoses	Clinical aspect	Dermoscopy	Pathology
Achantotic			
Hyperkeratotic			
Reticulated			
Clonal			
Irritated			

Source from Simionescu, O. *et al.* (2012)

References:

Greco, M.J, and Bhutta,B.S, (2023). Seborrheic Keratosis. *StatPearls[online]*. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK545285/#:-:text=There%20are%20various%20subtypes%20of,mu> (Accessed 16 Nov 2023)

Simionescu, O., Popescu, B. O., Costache, M., Manole, E., Spulber, S., Gherghiceanu, M., and Blum, A. (2012). Apoptosis in Seborrheic Keratoses: An Open Door to a New Dermoscopic Score. *J. Cell. Mol.Med.* Vol 16(6). Pp1223-12311. doi: 10.1111/j.1582-4934.2012.01558.x (Accessed 16 Nov 2023)

Bouhamed, M., Bacha, D., Abdelmoula, F., Ben Slama, S., Lahmar, A., Bouraoui, S., & Mzabi-Regaya, S. (2019). Clonal Seborrheic Keratosis: A Rare Skin Tumour. *Pan Afr Med J*,34(54). Doi: 10.11604/pamj.2019.34.54.13415 (Accessed 16 Nov 2023)

581 words

Reply



Re: Scenario 3
by [Jillian Simpson](#) - Thursday, 16 November 2023, 5:14 PM

I just wanted to add further to this discussion after seeing my first seborrhoeic keratosis a few months ago. Up until this point I had assumed that these lesions where associated with older skin, with the risk increasing as skin ages. I therefore thought it would be an unlikely diagnosis in a patient aged 36 years.

This patient had a brown papule with a smooth surface, the lesion appeared stuck on as though it

could be picked off. Given his age I was concerned he was too young and the diagnosis must be something else, he confessed to loving the sun and seeking a tan most summer, so I was worried it could be a malignant melanoma. On senior review and further assessment with dermatoscopy the diagnosis was confirmed as a seborrhoeic keratosis.

Some points of differentiation between the two: melanoma have a smooth surface and variability in elevation, colour and pigment density. Seborrhoeic keratosis has a stuck on appearance, presence of horny cysts on the surface and a uniform surface (Zaidi & Lanigan, 2010).

I wondered if others had seen this in anyone younger than 36 years?

References:

Zaidi, Z. Lanigan, S.W.(2010). Dermatology in Clinical Practice. Springer. London. P406

200 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:31 PM

Jillian, thank you for your post, SK is predominantly a condition of the middle aged but can be seen in patients as young as 15 especially if there is a family history of the condition. Please have a look at this article <https://jamanetwork.com/journals/jamadermatology/fullarticle/190329>

43 words

Reply



Re: Scenario 3

by [Jillian Simpson](#) - Friday, 17 November 2023, 12:36 PM

That is very helpful, thank you.

6 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:39 PM

You are welcome

3 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:20 PM

Xiao Liey, thank you for responding to Rahimas question and thank you for the images.

15 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:03 PM

Thank you Rahima, good post.

5 words

Reply

**Re: Scenario 3**by [Derek Kayanja \(Tutor\)](#) - Thursday, 16 November 2023, 7:13 PM

Hi Group,

Thanks to all who have submitted a post, this is as you have all correctly diagnosed a case of Seborrheic Keratosis. I shall let other members of the group submit their posts, and post my responses tomorrow. Very well done all of you !.

46 words

[Reply](#)**Re: Scenario 3**by [Siddharth Saluja](#) - Friday, 17 November 2023, 11:10 AM

Q1 and Q2

The observed abnormality bears a resemblance to seborrheic keratosis.

This is a frequent benign skin growth that presents itself as either wart-like or attached lesions, which can display a range of colors such as yellow, grey, brown, and black. (1)

The surface of these can be either smooth and waxy or covered in warts. (1)

Q3

Typically, the diagnosis of seborrheic keratosis relies on evaluating the condition through clinical assessment. However, I believe that utilizing dermatoscopy as a non-invasive diagnostic method would be beneficial since it enables a more detailed analysis of the skin and can aid in the further evaluation of the features of seborrheic keratoses.

We can use the following methods to treat: (2)

1)

Cryotherapy. Your healthcare provider will numb the skin and then use liquid nitrogen to freeze the growth. This will cause it to fall off within a few days or weeks.

2)

Electrodessication/Curettage. Your healthcare provider will numb the skin and then use a targeted electrocurrent to burn the seborrheic keratosis. They use a surgical instrument called a curette to scrape away the remains of the growth. Electrodessication and curettage are also sometimes used individually. The risk of scarring is generally low with both methods, but there is some wound care involved afterward. (2)

3)

Shave Excision. This is the preferred method when your healthcare provider wants to preserve a sample of the growth to analyze in the lab. After numbing the skin, your healthcare provider will carefully shave off the growth and smooth the skin underneath with a surgical curette.(3)

4)

Laser Therapy. Lasers offer an alternative to surgery by burning the growth, sterilizing the wound and sealing the tissue all at once. Laser therapy is quick, but the wound will be sore for a while afterward. Lasers are associated with good cosmetic results.(3)

References:

1)Cleveland Clinic. (n.d.). Seborrheic Keratosis: What Is It, Causes, Risks & Treatment. [online] Available at: <https://my.clevelandclinic.org/health/diseases/21721-seborrheic-keratosis>.

2)Greco, M.J., Mahabadi, N. and Gossman, W. (2020). Seborrheic Keratosis. [online] PubMed. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK545285/>.

3)Healthline. (2012). Seborrheic Keratosis: Risks, Diagnosis, & Treatment. [online] Available at: <https://www.healthline.com/health/seborrheic-keratosis>.

348 words

[Reply](#)**Re: Scenario 3**by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:03 PM

Thank you Siddharth, good post.

5 words

[Reply](#)**Re: Scenario 3**by [Siddharth Saluja](#) - Saturday, 18 November 2023, 8:04 AM

thanks Derek

2 words

[Reply](#)**Re: Scenario 3**by [Derek Kayanja \(Tutor\)](#) - Friday, 17 November 2023, 12:15 PM

Hi Group,

Thank you all who contributed to this scenario.

This is a case of Seborrheic keratosis (SK) the bedside test would be useful in confirming the diagnosis is Dermoscopy

The most appropriate management strategy for this patient is Reassurance and discharge

Dermoscopy clearly demonstrates the salient features including follicular openings, white horny cysts (also known as Milia-like cysts) and small comma vessels. Often the lesions are multiple and in this case asymptomatic.

Please visit this link for some more information on the dermoscopy of SK

https://dermoscopedia.org/Seborrheic_keratosis

87 words

[Reply](#)**Re: Scenario 3**by [Pooran Outar](#) - Saturday, 18 November 2023, 1:20 AM

The skin lesion seems to be verrucous, dark brown, waxy and firmly attached to the skin's surface.

Diagnosis: Seborrheic keratosis

A simple bedside examination using a hand lens, one can detect the existence of horn cysts or dark keratin plugs within the keratosis (Kim and Brian Jiang, 2016).

While SKs are usually harmless and symptom-free, different treatment options are available, and the choice depends on factors like lesion characteristics and individual preferences. **Treatment methods** include cryotherapy, which freezes lesions with liquid nitrogen, curettage and electrocautery involving scraping and burning, ablative laser surgery, scalpel shaving, and chemical peels. However, each method has drawbacks, such as potential pigment loss and scarring (The British Association of Dermatologists, 2022); (Oakley, 2016).

References:

- Kim, S.S. and Brian Jiang, S.I. (2016). *Seborrheic Keratoses*. [online] Plastic Surgery Key. Available at: <https://plasticsurgerykey.com/seborrheic-keratoses/> [Accessed 16 Nov. 2023].
- Oakley, A. (2016). *Seborrheic keratoses (brown warts, basal cell papillomas, seborrheic keratosis)*. [online] dermnetnz.org. Available at: <https://dermnetnz.org/topics/seborrheic-keratosis/>.
- The British Association of Dermatologists (2022). *British Association of Dermatologists*. [online] www.bad.org.uk. Available at: <https://www.bad.org.uk/pils/seborrheic-keratosis/>.

173 words

Reply



Re: Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Sunday, 19 November 2023, 9:05 AM

Pooran, thank you for your contribution

6 words

Reply

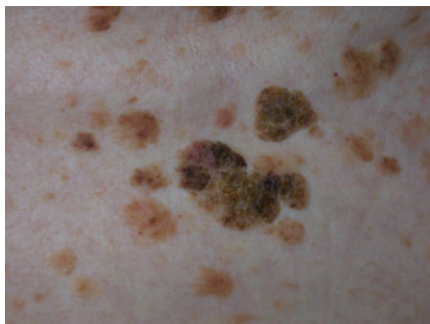


Re: Scenario 3

by [Arwaf Benrawwaf](#) - Monday, 11 December 2023, 4:37 PM

1. To confirm the diagnosis of the pigmented lesion on the patient's mid back, a skin surface microscopy or dermatoscopy would be beneficial. This technique permits a more thorough examination of skin lesions and can aid in distinguishing between harmless and cancerous lesions. (Lallas et al., 2013).

2. The magnified appearance of the lesion does not suggest irregular pigmentation, asymmetry, or an irregular border, which are features consistent with melanoma. Instead, it shows a well-demarcated, brownish-black lesion with a rough, warty surface, which is consistent with seborrheic keratosis (Lallas et al., 2013).



(Oakley, A. 2016)

3. Seborrheic keratosis is a benign lesion and does not require treatment unless it is causing symptoms such as itching or bleeding, or if it is cosmetically bothersome to the patient. In this case, the occasional sensation of itching reported by the patient may warrant treatment. Treatment options include cryotherapy, electrocautery, curettage, or shave excision (Greco, M.J.2023). However, given the location of the lesion on the mid back region, shave excision may be the most appropriate option. The patient should also be counseled on sun protection measures, including the use of sunscreen and protective clothing, to reduce the risk of further skin damage and development of additional skin lesions. Regular follow-up with a dermatologist is also recommended to monitor for recurrence or development of new lesions.

References:

- Lallas, A., Argenziano, G., Enrico Zendri, Moscarella, E., Longo, C., Grenzi, L., Pellacani, G. and Zalaudek, I. (2013). Update on non-melanoma skin cancer and the value of dermoscopy in its diagnosis and treatment monitoring. 13(5), pp.541–558. doi:<https://doi.org/10.1586/era.13.38>.

- Greco, M.J. and Bhutta, B.S. (2023). *Seborrheic Keratosis*. [online] PubMed. Available at: <https://pubmed.ncbi.nlm.nih.gov/31424869/> [Accessed 11 Dec. 2023].

-Oakley, A. (2016). *Seborrhoeic keratoses (brown warts, basal cell papillomas, seborrheic keratosis)*. [online] dermnetnz.org. Available at: <https://dermnetnz.org/topics/seborrhoeic-keratosis>.

297 words

Reply

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